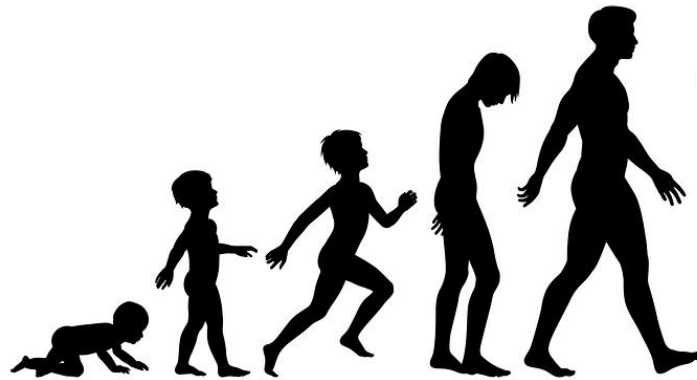


Session Objectives

- Identify how the stages of adolescent development contribute to certain behaviors and risk
- Understand important aspects of gathering a history from an adolescent with regards to confidentiality and the limits of confidentiality.
- Understand building rapport using a HEADS assessment, and risk assessment for problems common in the adolescent population
- Describe and understand adolescent pubertal physical development
- Understand the elements of an adolescent physical exam based on common conditions encountered in adolescence

- Adolescents
 - Transition from childhood to adulthood
 - Ages 11 to 21
 - The age at onset of puberty varies but the stages always go in the same sequence



Understand the Adolescent- Stages

Early (11-14yrs)

- Puberty starting, starting struggle for independence but still childish in many ways, same sex peer groups, starting interest in sexuality, concrete thinking (things black and white/ right and wrong)

Middle (15-17yrs)

- Continued physical changes, independence progressing, love/passions, family conflict highest, mixed peer groups, maturing thought process

Late (18-21yrs)

- more mature self identity, thinking and independence, peers and family dynamics change, mature relationships, can think more about future and others, understand life may be grey, not as simple as black and white/ right and wrong

Build Rapport with Adolescent AND Parent

- Introductions
- Ask teen and parents if any concerns
 - Chit/ Chat- no confidential Questions yet
 - +/- Physical exam
 - Get a feel for the Temp in the room
 - Get a sense of cognitive and pubertal development
- At some point ask parent to leave the room. "I would like to talk to XXX alone for a bit, would you mind stepping out of the room"
- The younger the patient the more mindful you need to be esp 11 and 12 y/o



Confidentiality Statement

- When alone, ‘whatever is discussed will be kept confidential, I don’t tell your parents anything you don’t want me to. The only exception is if there is a danger to yourself or someone else.’

Confidentiality Limits

- Never make confidentiality unlimited
- Your patient needs to know that they can tell you things in confidence but they have to know that there are exceptions
 - Threat to others
 - Threat to self

STD's and Pregnancy stays confidential

H.E.A.D.S Assessment- “Interview ALONE”

Confidentiality statement

Home

Education, **E**mployment

Activities

Drugs, **D**rinking, **D**iet, **D**epression

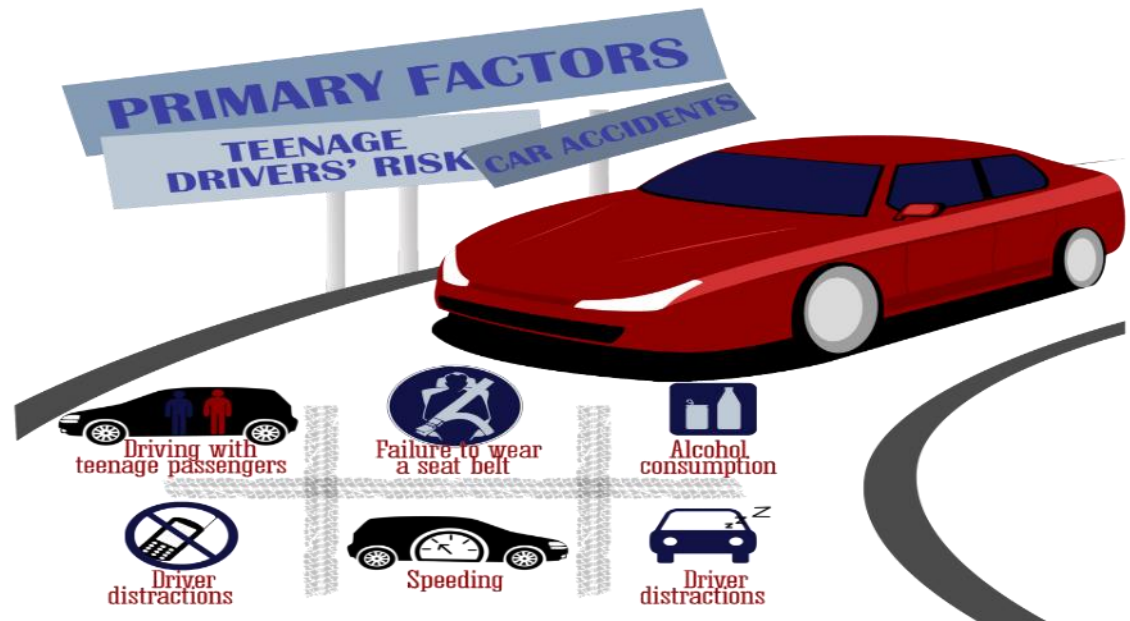
Sex, **S**moking, **S**uicide

- Ask and get specifics
 - How much, how often, situation? passed out?
 - Marijuana? Any Pills? Ecstasy/ Molly? Etc
 - Need to use to feel normal?
 - How do you get the money?
 - CRAFFT Screening
 - CAGE Questionnaire for alcohol

- **MVA #1 Killer of Teenagers**

- In 2014, 2,270 teens in the United States ages 16–19 were killed and 221,313 were treated in emergency departments for injuries suffered in motor vehicle crashes.¹ That means that six teens ages 16–19 died every day from motor vehicle injuries

- **Driving while intoxicated, impaired, distracted**



PHQ-2 screening for Depression

Patient Health Questionnaire-2 (PHQ-2)

 Share

The PHQ-2 inquires about the frequency of depressed mood and anhedonia over the past two weeks. The PHQ-2 includes the first two items of the PHQ-9.

- The purpose of the PHQ-2 is to screen for depression in a “first-step” approach.
- Patients who screen positive should be further evaluated with the PHQ-9 to determine whether they meet criteria for a depressive disorder.

Over the **last 2 weeks**, how often have you been bothered by the following problems?

Not at all

Several days

More than half the days

Nearly every day

1. Little interest or pleasure in doing things

☐

0

☐

+1

☐

+2

☐

+3

2. Feeling down, depressed or hopeless

☐

0

☐

+1

☐

+2

☐

+3

PHQ-2 score obtained by adding score for each question (total points)

Interpretation:

- A PHQ-2 score ranges from 0-6. The authors identified a score of 3 as the optimal cutpoint when using the PHQ-2 to screen for depression.
- If the score is 3 or greater, major depressive disorder is likely.
- Patients who screen positive should be further evaluated with the [PHQ-9](#), other diagnostic instruments, or direct interview to determine whether they meet criteria for a depressive disorder.

Suicide

Recognize increasing likelihood of action

- Comorbid conditions (mental illness, depression)
- Gender identity/other factors (bullying)
- Incarceration
- Self destructive thoughts
- Prior attempts
- Substance abuse (ETOH, Drugs)
- Giving away possessions
- Access to lethal means
- Hallucinations (commands)
- Signif changes/ stressors/ inciting events
- Recent death or suicide of friend/ relative

Assess Suicidal Ideation/ Homicidal ideation



Suicidal Ideation- assess severity, plan, previous attempts

- Educate, discuss Action plan if thoughts worsen, persists (988)

Homicidal Ideation- anger, plan, obsession

Imminent Danger to self or others: can break confidentiality

Try to have adolescent agree to discuss with parent so don't have to break Confidence if SI is high risk

More questions...dig deeper if indicated

- Abuse- physical, sexual
- Intentional injury- cutting, burning
- Eating Disorders
- Inappropriate 'adult' encounters

Assessing Pubertal Development

- Breast/ genitalia exam with chaperone (parents aren't chaperones)
- Puberty onset
 - 8 to 13 years in Females (Average age is 11 years)
 - 9 to 14 in males (Average age is 12 years)
 - Males on ave usually start puberty 2 yrs later than females
- Growth spurt for girls happens early in puberty (tanner 2-3), menses tanner 4,6, Boys happens later in puberty.

- Vaccines (HPV, Tdap, MCV4, Men B)
- Lab work?
 - Lipids testing, Anemia?, PCOS?/ menstrual issues?/ obesity?/ STD testing?
- Referral?
- Medications?
- Imaging?
- Anticipatory Guidance
- Words of wisdom (for both patient and Parents)
- follow-ups

- **Primary care is primary provider**
 - **Screening, often start medications if needed**
- Social work
- Guidance counselor
- Psychologists
- Psychiatrists
- Adolescent specialists
- Hospital for acute care

Early (11-14yrs)

- Puberty starting, starting struggle for independence but still childish in many ways, same sex peer groups, starting interest in sexuality, concrete thinking- (things black and white/ right and wrong)

Discussions:

- Don't dismiss their struggle
- Puberty and changes
- Discuss in concrete terms, minimize analogies/ abstractions
- Safety: Most likely to act without thinking...social media, sex, drugs, vaping
- **Plant seeds**

Middle (15-17 yrs)

- Continued physical changes, independence progressing, love/passions, family conflict highest, mixed peer groups

Discussions:

- Still concrete but may have higher level discussion
- Friends/family/relationship changes
- Love interests/ more mature relationships
- Safety: +driving
- Promote independence and corresponding responsibilities (job)
- Water, stake, plant more seeds

Late (18-21)

- more mature self identity, thinking and independence, peers and family dynamics change, mature relationships, can think more about future and others, understand life may be grey, not as simple as black and white/ right and wrong

Discussions:

- More mature conversation
- Less lecturing, More refinement of ideas, guidance, suggestions
- Guide specific goals and plans about future
- Water, admire the plant, plant more

- Varies by region/ pediatricians/ patients
 - Considerations-
 - adult medical problems
 - Pediatric medical problems esp syndromes, dev delay
 - Continuity of care
- Most important for adolescents with **SPECIAL HEALTHCARE NEEDS**
 - IMPROVES THEIR HEALTH OUTCOMES
 - IMPROVES QUALITY OF LIFE
 - PREVENTS LOSS OF A MEDICAL HOME
 - IMPROVES CHRONIC DISEASE CONTROL

Documenting/Recording a Adolescent Exam

- Please refer to Bates guide to physical examination and history taking (11th ed.). Pages 872-875
- The adolescent **SUBJECTIVE** is more detailed than previous exams given the information you gather from TWO HISTORIANS (THE PATIENT AND THE PARENT). The Objective, Assessment and Plan are the same. Note if discussed alone with patient, document chaperone present for breast/genitalia exam

Summary Slide

- Tailor your questions and discussions based on adolescent developmental stages- Early, Mid, Late
- Non-accidental injury, Suicide, homicide are top causes of mortality, then neoplasm and Heart Disease
- Build rapport with both first, then talk alone and confidentiality/disclaimer statement. Understand and state limits of confidentiality
- HEADS assessment
 - Start with less intrusive H,E,A and look for red flags
 - More personal D and S after rapport built
- Screens: PHQ2,9,CRAFFT,CAGE,GAD2,7
- Course of action if Suicidal ideation, 988, counseling, Hosp if imminent
- Pertinent Physical findings in an adolescent
 - Scoliosis 7degrees on scoliometer need further eval, risk during growth spurt)
- First sign of puberty in female is breast buds (age 8-13), first in males is scrotal enlargement (age 9-14)

Arkansas Law- Adolescent Right to Care

- ARK. CODE ANN. § 20-9-602 (2012). CONSENT GENERALLY
 - (section 7) Any unemancipated minor of sufficient intelligence to understand and appreciate the consequences of the proposed surgical or medical treatment or procedures, for himself or herself (can consent to treatment)
- ARK. CODE ANN. §20-16-508 (2012). CONSENT BY MINOR 24
 - (a)(1) When a minor who believes himself or herself to have a sexually transmitted disease consents to the provision of medical care or surgical care or services by a hospital or public clinic or consents to the performance of medical care or surgical care or services by a physician who is licensed to practice medicine in this state, the consent: (A) Is valid and binding as if the minor had achieved his or her majority; and (B) Is not subject to a later disaffirmance by reason of his or her minority. (2) **The consent of a spouse, parent, guardian, or any other person standing in a fiduciary capacity to the minor shall not be necessary in order to authorize hospital care or services or medical or surgical care or services to be provided to the minor by a physician licensed to practice medicine.** (b) Upon the advice and direction of a treating physician or in the case of a medical staff any one (1) of them, a physician or member of a medical staff may inform the spouse, parent, or guardian of any minor as to the treatment given or needed but shall not be obligated to do so. The information may be given to or withheld from the spouse, parent, or guardian without the consent and over the express objection of the minor.

NEW YORK LAW- Minors Right to Medical Care

- **N.Y. PUB. HEALTH LAW § 2504 (2012). ENABLING CERTAIN PERSONS TO CONSENT FOR CERTAIN MEDICAL, DENTAL, HEALTH AND HOSPITAL SERVICES**
 - 1. Any person who is eighteen years of age or older, or is the parent of a child or has married, may give effective consent for medical, dental, health and hospital services for himself or herself, and the consent of no other person shall be necessary.
 - 2. Any person who has been married or who has borne a child may give effective consent for medical, dental, health and hospital services for his or her child. Any person who has been designated pursuant to title fifteen-A of article five of the general obligations law as a person in parental relation to a child may consent to any medical, dental, health and hospital services for such child for which consent is otherwise required which are not: (a) major medical treatment as defined in [subdivision \(a\) of section 80.03 of the mental hygiene law](#); (b) electroconvulsive therapy; or (c) the withdrawal or discontinuance of medical treatment which is sustaining life functions.
 - 3. Any person who is pregnant may give effective consent for medical, dental, health and hospital services relating to prenatal care.
 - 4. Medical, dental, health and hospital services may be rendered to persons of any age without the consent of a parent or legal guardian when, in the physician's judgment an emergency exists and the person is in immediate need of medical attention and an attempt to secure consent would result in delay of treatment which would increase the risk to the person's life or health.
 - 5. Where not otherwise already authorized by law to do so, any person in a parental relation to a child as defined in section twenty-one hundred sixty-four of this chapter and, (i) a grandparent, an adult brother or sister, an adult aunt or uncle, any of whom has assumed care of the child and, (ii) an adult who has care of the child and has written authorization to consent from a person in a parental relation to a child as defined in section twenty-one hundred sixty-four of this chapter, may give effective consent for the immunization of a child. However, a person other than one in a parental relation to the child shall not give consent under this subdivision if he or she has reason to believe that a person in parental relation to the child as defined in section twenty-one hundred sixty-four of this chapter objects to the immunization.
 - 6. Anyone who acts in good faith based on the representation by a person that he is eligible to consent pursuant to the terms of this section shall be deemed to have received effective consent.